H Alabama Medicaid Physician Administered Drugs

Effective October 1, 2010, the NDC is required on all physician-administered drugs in J, S, and Q code ranges. Physician-administered drugs include any covered outpatient drug billed either electronically or on paper CMS-1500 or UB-04 claim forms.

H.1 Policy

H.1.1 Injections

Medicaid covers physician administered drugs when billed by a physician using the new list of approved HCPCS codes.

The HCPCS drug codes are intended for use in physician offices and outpatient billing of manufactured medications given in each respective place of service. The Alabama Medicaid Agency only reimburses for compounded medications by the billing of NDC numbers through the Pharmacy Program directives.

Appropriate administration code(s) in the Current Procedural Terminology (CPT) Manual may be billed in addition to the HCPCS drug codes and office visit codes for the same date of service. Please refer to the following section "Evaluation and Management Codes Billed in Conjunction With Drug Administration Codes" for details concerning office visits, chemotherapy administration, hydration therapy and chemotherapy, and date specific changes.

Pricing of Physician Administered Drugs

The Alabama Medicaid Agency adopted Medicare's Drug Pricing Methodology using the Average Sale Price (ASP) for HCPCS injectable drug codes. Physician Administered Drugs (PADs) are reimbursed at a rate of Average Sales Price (ASP) +6%. For PADs that do not have a published ASP, the reimbursement is calculated based on the published compendia pricing such as Wholesale Acquistion Cost (WAC). For PADs administered by 340 entities, payment should not exceed the entity's actual acquisition cost for the drug.

Compound Drugs for Non-Pharmacy Providers

The compound drug must not be commercially available, and the active ingredient of the compound drug must follow coverage policy of drugs (FDA approved, non-Drug Efficacy Study Implementation (non-DESI), not obsolete, etc.).

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When a provider administers a drug that must be purchased from a compounding pharmacy because it is no longer commercially available (e.g. due to the manufacturer no longer marketing the product), the applicable claim form may be submitted for consideration of payment. The billed amount should represent the lesser of the actual acquisition cost for the drug or Medicaid rate on file (ASP CMS pricing) at the time of service.

When billing the HCPCS code for a purchased compounded drug, only one NDC can be used per procedure code. Providers must use the HCPCS procedure code, billing units and corresponding covered NDC number on the claim form; for example, J1100 Injection, dexamethasone sodium phosphate, 1 mg. The NDC billed should be the one that represents the drug as described in the HCPCS code definition, in this case dexamethasone sodium phosphate. See the section entitled "Calculation of Billing Units and Wastage" for information on calculating billing units.

The Agency does not reimburse providers for prescription compounding time or non-covered ingredients used in the compounding process.

Mandatory National Drug Codes (NDC) for ALL Physician Administered Drugs

Effective October 1, 2010, the NDC number is mandatory on **ALL** physician-administered drugs in the following ranges: J0000 – J9999, S0000 – S9999, and Q0000 – Q9999. Physician-administered drugs include any covered outpatient drug billed either electronically or on a paper CMS-1500 or UB-04 claim forms. This is for both straight Medicaid and Medicare/Medicaid crossover claims. The 11-digit NDC submitted must be the actual NDC number on the package or container from which the medicine was administered. The NDC is a number that identifies a prescription drug.

This requirement applies to:

- All fee-for-service providers who bill physician-administered drug codes
- HCPCS codes in the ranges J0000 – J9999, S0000 – S9999, and Q0000 – Q9999.
- Both electronic and paper submissions

This requirement does **NOT** apply to:

- 340B Providers enrolled on the HHS website
- Vaccines or other drugs in the CPT code ranges 01000 – 99999.
- HCPCS that do not have an NDC
- HCPCS that are considered devices
- HCPCS that are considered radiopharmaceuticals
- Providers paid on a per diem, encounter, or other type of rate, which includes, but may not be limited to:
    - Inpatient Hospitals

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- Nursing Facilities
- Federally Qualified Health Centers
- Rural Health Centers
- Ambulatory Surgical Centers
- Home Health Agencies

To identify if a product is a drug, look for these three items:

1. NDC - Number located on the package or container of the drug
2. Lot and Expiration Date - All drugs have both a lot number and expiration date on the vial or container
3. Legend - This refers to statements such as, “Caution; Federal law prohibits dispensing without prescription, “Rx only” or similar words. All prescription drugs have these types of statements

As this process is to facilitate Medicaid drug rebates from manufactures for physician-administered drugs, providers are required to utilize drugs manufactured by companies who hold a federal rebate agreement. These NDCs will be the only ones Medicaid will cover for payment. A link to a list of those drug manufacturers who hold a federal rebate agreement, as well as their labeler codes (the first 5 digits of the NDC number), are available on the Medicaid website at: https://medicaid.alabama.gov/content/4.0_Programs/4.3_Pharmacy-DME/4.3.2_Billing_Policy_Info.aspx. Select the “covered labelers (manufacturers) available from CMS” link.

The Alabama Medicaid Agency implemented a Drug Lookup System effective October 5, 2010. The system allows non-pharmacy providers needing NDC information for the billing of HCPCS codes to search for drugs by drug name or NDC and will display coverage information. Providers can access the Drug Lookup feature by visiting the Alabama Medicaid website and clicking on the following link: https://www.medicaid.alabamaservices.org/ALPortal/NDC%20Look%20Up/tabId/39/Default.aspx

Please note that the information found on the drug look up website applies to pharmacy claims only (i.e: pricing, PA requirements, and maximum quantity limits).

Any questions should be directed to the Provider Assistance Center at 1-800-688-7989.

Multiple NDCs for a Single HCPCS Drug Code

At times it may be necessary for providers to report multiple NDCs for a single procedure code. If two or more NDCs are to be submitted for a procedure code, the procedure code must be repeated on separate lines for each unique NDC. On the first line, the procedure code, NDC and procedure quantity are reported with a **KP modifier** (first drug of a multi drug). On the second line, the procedure code, NDC and procedure quantity are reported with a **KQ modifier** (second/subsequent drug of a multi drug). When reporting more than two NDCs per procedure code, the KQ modifier is also used on the subsequent lines.

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Unclassified Drugs

A provider who administers a physician drug not listed should use the following J codes:

- J3490 - Unclassified Drugs
- J3590- Unclassified Biologics
- J9999 - Not otherwise classified, antineoplastic drugs.

The claim must be sent on paper with a description of the drug attached. Providers should submit an original red drop-out ink CMS 1500 claim with the complete name of the drug, total dosage that was administered and a National Drug Code (NDC) number. Please be sure to search the Physician Drug List to see if the drug is possibly under a generic name. The claims containing the unclassified procedure code must be sent to: Gainwell, Attn: Medical Policy, PO Box 244032, Montgomery, AL 36124-4032. Gainwell will determine the price of the drug.

The Alabama Medicaid Agency supports the avoidance of wasted (discarded) medicine whenever possible. Medicaid accepts the use of modifier JW on a second line item to indicate the wasted (discarded) amount of medication.

Breathing/Inhalation Treatment: J2545 Pentamidine Isethionate (Nebupent)

Current coverage policy for breathing or inhalation treatments utilizing drugs such as Albuterol does not allow for the drug to be billed for separately as it is considered a component of the treatment charge. The exception to this policy is the administration of Pentamidine Isethionate.

Pentamidine isethionate (J2545), given by inhalation, is an anti-microbial agent specifically indicated for the prevention of Pneumocystis carinii pneumonia (PCP) in high-risk HIV infected patients. Administration of Pentamidine is done via the Respirgard II nebulizer which utilizes a series of one-way valves and a filter to minimize the release of aerosol droplets into the air. CPT code 94642 (aerosol inhalation of Pentamidine for pneumocystis carinii pneumonia treatment or prophylaxis) is the appropriate code to bill for administration of the drug. This therapy is generally given monthly and given in the hospital or clinic/office by a health care professional. The administration code does not include the cost of the drug. The inhalation drug code and the administration of the drug should both be reported on the same claim, same date of service.

340B Drug Pricing

The Veterans Health Care Act of 1992 enacted section 340 B of the Public Health Services Act, "Limitation on Prices of Drugs Purchased by Covered Entities". This Section provides that a manufacturer who sells covered outpatient drugs to eligible 340B entities must sign a pharmaceutical pricing agreement with the Secretary of Health and Human Services in which the manufacturer agrees to charge to Medicaid a price for covered outpatient drugs that will not exceed the average manufacturer price decreased by a rebate percentage.

Eligible 340B entities are defined in 42 U.S.C. are defined in 42 U.S.C. § 256b(a)(4).

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When an eligible 340B entity, other than a disproportionate share hospital, a children's hospital excluded from the Medicare prospective payment system, a free-standing cancer hospital exempt from the Medicare prospective payment system, sole community hospital, rural referral center, or critical access hospital, submits a bill to the Medicaid Agency for a drug purchased by or on behalf of a Medicaid recipient, the amount billed shall not exceed the entity's actual acquisition cost for the drug, as charged by the manufacturer at a price consistent with the Veterans Health Care Act of 1992, plus the dispensing fee established by the Medicaid Agency.

A disproportionate share hospital, children's hospital excluded from the Medicare prospective payment system, free-standing cancer hospital exempt from the Medicare prospective payment system, sole community hospital, rural referral center, or critical access hospital may bill Medicaid the total charges for the drug. As manufacturer price changes occur, the entities must ensure that their billings are updated accordingly.

Eligible 340B entities are identified on the Department of Health and Human Service's website. These entities shall notify Medicaid of their designation as a 340B provider.

Audits of the eligible 340B entities' (claims submissions and invoices) will be conducted by the Medicaid Agency. Eligible 340B entities, other than the providers listed above, must be able to verify acquisition costs through review of actual invoices for the time frame specified. Charges to Medicaid in excess of the actual invoice costs will be subject to recoupment by the Medicaid Agency in accordance with Chapter 33 of the Administrative Code.

Medicare/Medicaid Drugs

Medicare Part B covers some drugs in a physician's office. If the recipient is dually eligible for Medicare and Medicaid, the drug code as required by Medicare should be billed first to Medicare. The claim should crossover to Medicaid for consideration of payment. If the claim does not crossover to Medicaid, providers will need to submit the appropriate HCPCS code to Medicaid on a Medical Medicaid/Medicare Related Claim (aka Crossover Form 340) with Medicare allowance/payment/coinsurance/ and deductible.

Medicare Part D drugs are a pharmacy benefit and should not be billed to Medicaid by physicians or outpatient facilities. Part D drugs are billed to Medicare on a pharmacy claim with the NDC number.

Not all drugs listed on the Physician Drug Fee Schedule are considered Part B drugs. Self Administered drugs are generally considered non-covered for Part B benefits. Coverage of Physician Drugs may be found on Medicaid's website at www.medicaid.alabama.gov or by AVRS or Provider Assistance Center at 1-800-688-7989.

Site-Specific Injections

Both the relevant CPT and J codes are billed. For example, a subconjunctival injection to the eye would be billed as 68200 (CPT) with a separate J code for the drug; thus, site specific injections are submitted as two lines.

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EVALUATION AND MANAGEMENT CODES BILLED IN CONJUNCTION WITH DRUG ADMINISTRATION CODES

Effective for Dates of Service 01/01/2006 and Thereafter

When an Evaluation and Management service is provided *and* a Drug Administration code (96372, 96373, 96374, 96375 and 96376) is provided at the same time, the E & M code, Drug Administration Code, and the HCPCS Code for the drug may be billed. A **Significant Separately Identifiable Service** must be performed in conjunction with the Drug Administration code for consideration of payment for the Evaluation and Management Code. A **Modifier 25** must be appended to the E&M service for recognition as a "**Significant Separately Identifiable Service".** Medical Record documentation must support the medical necessity of the visit as well as the level of care provided.

However, when no **Significant Separately Identifiable** E & M service is actually provided at the time of a Drug Administration, an E & M code should not be billed. In this instance, the Drug Administration Code and the HCPCS Code for the drug may be billed. An example of this is routine monthly injections like B-12, iron, or Depo-Provera given on a regular basis without a **Significant Separately Identifiable** E & M service being provided.

When an Evaluation and Management service is provided and an Administration Code for Hydration (96360, 96361), Therapeutic, Prophylactic, and Diagnostic Infusion (96365, 96366, 96367 and 96368) and Chemotherapy Administration Code (96401-96542) is provided at the same time/encounter, the E&M code and Administration code may be billed. A **Significant Separately Identifiable Service** must be performed in conjunction with these administration codes for consideration of payment for the Evaluation and Management Code. A **Modifier 25** must be appended to the E & M service for recognition as a **"Significant Separately Identifiable Service".** Procedure Codes 99211 will not be allowed with Modifier 25 or in conjunction with the administration codes for the same date of service. Medical record documentation must support the medical necessity and level of care of the visit. These services are subject to post payment review.

NOTE:

In accordance with Section 5042 of the SUPPORT Act, effective October 1, 2021, prescribers of Medicaid eligible recipients are required to check the Alabama PDMP (Prescription Drug Monitoring Program) prior to prescribing a Schedule II controlled substance. If the prescriber does not check the PDMP, the prescriber is required to document the reason in the medical record. Exclusions to this requirement include prescriptions written for hospice patients, patients with an active cancer diagnosis, residents of a long-term care nursing facility, and children under the age of 18 (Schedule II prescription for ADHD only).

Chemotherapy Injections

Alabama Medicaid has established the following new guidelines that should be utilized by physicians when billing for administration codes.

- For non-chemotherapy injections, services described by CPT codes 96372, 96374, and 96375 may be billed in addition to other

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physician fee schedule services billed by the same provider on the same day of service.

- For IV infusions and chemotherapy infusions, if a significant separately identifiable E & M service is performed, the appropriate E & M CPT code should be reported utilizing modifier 25.
- When administering multiple infusions, injections, or combinations, only one "initial" drug administration service code should be reported per patient per day, unless protocol requires that two separate IV sites must be utilized. The initial code is the code that best describes the service the patient is receiving and the additional codes are secondary to the initial code.
- "Subsequent" drug administration codes, or codes that state the code is listed separately in addition to the code for the primary procedure, should be used to report these secondary codes. If an injection or infusion is of a subsequent or concurrent nature, even if it is the first such service within that group of services, then a subsequent or concurrent code from the appropriate section should be reported.
- If the patient has to come back for a separately identifiable service on the same day, or has 2 IV lines per protocol, these services are considered separately billable with a modifier 76.
- Medicaid will not pay for chemotherapy administration in a hospital setting, and claims for these codes with modifier 26 will not be recognized.

Please refer to Chapter 19 (Hospitals) for details on chemotherapy administration and infusion therapy.

Drugs Requiring Prior Authorization

EXAMPLE:

Drugs requiring a prior authorization must be approved through Acentra Health prior to administering and billing. Acentra Health may be contacted at 1-800-748-0130. The Prior Authorization forms are located on the Medicaid website at www.medicaid.alabama.gov. Please refer to the Physician Drug Fee Schedule to confirm prior authorization requirements.

Allergy Treatments

Physicians may bill for antigen services using only the component codes (i.e., the injection only codes 95115 or 95117) and/or the codes representing antigens and their preparation (i.e., codes 95144 through 95170). Physicians providing only an injection service must bill for only code 95115 or code 95117. Professional services for allergen immunotherapy multiple injections (procedure codes 95117 and 95125) should be billed using only one unit. Effective April 1, 2003, the Agency will deny claims for these procedure codes when more than one unit is billed.

Physicians providing only the antigen/antigen preparation service would bill the appropriate code in the range of 95144 through 95170. Physicians providing both services would bill for both services. This includes allergists who provide both services through the use of treatment boards.

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Physicians will no longer use the "complete" service codes, and instead must bill for both the injection and the antigen services separately, even though the current CPT definitions of the antigen codes refer to vials and the physicians using treatment boards do not create vials.

Procedure codes 95144 - 95170 are used for the provision of single or multi-dose vials of allergenic extract for single patient use only. These procedures should only be billed at the time that these vials are supplied to the patient.

For the provision of allergen immunotherapy, Medicaid allows billing for the allergen at the time an individual vial is first used for a patient, but not for the entire amount of allergen/dilution prepared for the patient at once as this would likely exceed the maximum number of allowed units.

Procedure Code 95165 represents the preparation of vials of non-venom antigens. The reimbursement for procedure code 95165 is based on preparing a vial containing a mixture of all the appropriate antigens plus diluents and calculating the billing units in the vial.

Therefore, one (1) cc equals one (1) billing unit. The actual number of doses received by a patient may differ significantly from the number of billing units.
When a multidose vial contains less than 10cc, physicians should bill Medicaid for the number of 1cc billing units that may be removed from the vial. If a physician prepares two 10cc vials containing **different allergens**, he/she may bill Medicaid for a total of 20 billing units (10 billing units per vial).

The maximum number of billable units (three-10cc vials) for procedure code 95165 was set as “30” by the National Correct Coding Initiative (NCCI). If multiple vials are prepared at one time, each vial should be billed when that vial is opened for use for the patient. Administration of vaccine may continue to be billed as each dose is given in the physician’s office. Medical record documentation must clearly support the treatment plan, each vial used, antigens, dosage, and changes in the treatment regime.

Claims exceeding 30 billing units (such as three 10cc vials containing different allergens) will require manual processing by sending a clean claim with medical justification, medical records, and supporting fact based documentation to:

Alabama Medicaid Agency
P.O. Box 5624
Montgomery, Alabama, 36104
Attention: Physicians Programs

Calculation of Billing Units and Wastage

HCPCS code for J0587 reads “per 100 units”. Therefore, 100 units of J0587 will equal one billing unit. However, because of the expense of the drug, physicians are encouraged to schedule patients in a manner that they can use botulinum toxin most efficiently. For example, a physician schedules three patients requiring botulinum toxin type A on the same day within the designated shelf life of the drug (shelf life is four hours). The physician administers 30 units to all three patients and bills 30 units for the first two patients and 40 units for the last patient. The physician would bill

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40 units for the last patient because the patient received 30 units but the physician had to discard 10 units.

HCPCS code for J0585 reads “per unit”. Therefore, this code requires the units of service on the claim to reflect the number of units used. However, if a physician must discard the remainder of a single dose vial (sdv) after administering it to a patient, the Agency will cover the amount of the drug discarded along with the amount administered. For example, a physician administers 15 units of botulinum toxin type A and it is not practical to schedule another patient who requires botulinum toxin. Situations that are impractical to schedule another patient include (a) it is the first time the physician has seen the patient and did not know the patient’s condition or (b) the physician has no other patients who require botulinum toxin injections.

Documentation requirements must include the exact dosage of the drug given and the exact amount of the discarded portion in the patient’s medical record as well as the corresponding diagnosis. However, if no benefit is demonstrable by two sets of injections, further injections will not be considered medically necessary.

Modifier JW

The Agency supports the avoidance of wasted (discarded) medicine whenever possible. Medicare requests the use of modifier JW on a second line item to indicate the wasted (discarded) amount of medication. Medicaid accepts the use of modifier JW, but total units must not exceed maximum number of allowed units.

Units of Service

Physician drug maximum number of units allowed are calculated based on a “per dose” basis, and by the narrative description of the HCPCS code. Some dosages are inherent in the narrative description of the codes and will assist in determining the number of units to file. When administering a lesser or greater dosage than the narrative description providers should round the billing unit up to the closest amount charted. For example, J0290, Ampicillin, up to 500 mg:

If administering 1000mg, bill 2 units

750 mg, bill 2 units
500 mg, bill 1 unit
125 mg, bill 1 unit

Exception: Bicillin CR and Bicillin LA

Effective January 1, 2011, Bicillin CR and Bicillin LA will be priced on a 100,000 unit per ML basis. As well, the HCPCS codes have been condensed into two vs. six codes:

If administering Bicillin CR, bill J0558 (replaces J0530, J0540 and J0550)

If administering Bicillin LA, bill J0561 (replaces J0560, 05670 and J0580)

One of the two HCPCS codes should be chosen based on the drug description. The number of billing units would then be derived by dividing the dosage by 100,000 units. Fractions of billing units are rounded up to the next whole unit.

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Example: If the dosage of Bicillin LA is 1,800,000 units, choose the appropriate procedure code. In this case procedure code J0561 is the appropriate code to be used. Next, take the dosage given (1,800,000 units) and divide by 100,000 units to obtain the billing units. This dosage would yield 18 billing units (1,800,000 /100,000 = 18 units) for code J0561.

Flu Vaccination

For influenza vaccine procedure codes covered under the Vaccines for Children (VFC) Program to eligible children less than nineteen years of age, refer to Appendix A, Well Child Check-Up (EPSDT).

Influenza vaccines are covered fee-for-service (vaccine medication) for ages nineteen and above. For more specific information on coverage, you may call the Provider Assistance Center at 1 (800) 688-7989.

Vaccines for Children (VFC)

The Vaccines for Children (VFC) Program offers free vaccines to qualified health care providers for children who are 18 years of age and under who are Medicaid eligible, uninsured, American Indian or Alaskan Native, or the under insured. Providers must be enrolled in the VFC Program to receive any reimbursement for the administration of immunizations provided to recipients 0-18 years of age. The Alabama Department of Public Health administers this program.

Medicaid tracks usage of the vaccine through billing of the administration fee using CPT codes. Refer to Appendix A, Well Child Check-Up (EPSDT), for covered CPT codes.

ImmPRINT Immunization Provider Registry

The Alabama Department of Public Health has established a statewide immunization registry. Please visit their website at https://siis.state.al.us for more information.

Stand-alone Vaccine Counseling Codes

The following procedure codes may be used for stand-alone vaccine counseling for recipients under 21 years of age.

Procedure Code	Procedure Code Description
G0312	Immunization counseling by a physician or other qualified health care professional when the vaccine(s) is not administered on the same date of service for ages under 21, 5 to 15 mins time.
G0313	Immunization counseling by a physician or other qualified health care professional when the vaccine(s) is not administered on the same date of service for ages under 21, 16-30 mins time.
G0314	Immunization counseling by a physician or other qualified health care professional for COVID-19, ages under 21, **16-30** mins time.
G0315	Immunization counseling by a physician or other qualified health care professional for COVID-19, ages under 21, **5-15** mins time.

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- These procedure codes are to be used when ONLY providing vaccine counseling to a recipient under age 21, and the recipient does NOT receive a vaccine on the day of service.

NOTE:
G0315 is for 5-15 minutes of time; whereas, G0314 is for 16-30 minutes of time.

Adult Immunizations

For Medicaid eligible recipients 19 years or older, vaccines approved by the Advisory Committee on Immunizatioon (ACIP) will be paid by Medicaid without cost sharing to the recipient. Vaccines are reimbursable on a fee-for-service basis using the appropriate CPT code. The administration fee is included in the price of the vaccine. The administration fee may be billed separately if an office visit is not billed.

Provider should bill only the vaccine administration fee if the vaccine is provided at no cost to the provider from federal resources.

If a 90000s CPT vaccine code does not have a rate, a manual review will need to be submitted. A paper claim should include the following:

- Form 402
- A clean red drop-out ink CMS Form 1500 containing the vaccine procedure code
- Documentation of the total dosage that was administered
- Invoice with the complete name of the drug and a National Drug Code (NDC) number.

Claims may be submitted to:

Alabama Medicaid Agency
Attn: Dental / EPSDT / Physician Division
501 Dexter Avenue
P.O. Box 5624
Montgomery, AL 36103-5624

Radiopharmaceutical Drugs (Invoice Priced)

A provider who administers a Radiopharmaceutical drug not priced on the www.palmettogba.com website should use the following criteria:

- The claim must be sent on paper with a description of the drug attached.
- Providers should submit a red drop-out ink claim with the complete name of the drug, total dosage that was administered and a National Drug Code (NDC) number.
- The claims containing the radiopharmaceutical procedure code must be sent to:

  Gainwell, Attn: Medical Policy
  PO Box 244032
  Montgomery, AL 36124-4032

Gainwell will determine the price of the drug.

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H.2 Physician Drug Fee Schedule

Physician Adminstered Drugs are those that are administered in the Physician's office or outpatient facility. A covered outpatient drug is broadly defined as a drug that may be dispensed only upon prescription and is approved for safety and effectiveness by the FDA. Physician administered drugs are not restricted to injectable drugs only but include any drug regardless of the method of administration.

The inclusion or exclusion of a procedure code does not imply Medicaid coverage, reimbursement, or lack thereof. To inquire regarding any restrictions/limits on these procedure codes, please consult the Provider Assistance Center at 1-800-688-7989 or AVRS at 1-800-727-7848. The pricing file must be verified to determine coverage and reimbursement amounts.

The Physician Drug Fee Schedule is located on the Alabama Medicaid website and can be accessed by clicking the following link: http://medicaid.alabama.gov/content/Gated/7.3G_Fee_Schedules.aspx

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